جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : اكرام ابراهيم عبده محمد
المحافظة : القاهره **الحي / المدينة :** عابدين **العنوان :** ....
رقم البطاقة : 26501260100308 **رقم التليفون :** 01265986763 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 26-01-1965 **المهنة :** اعمال صيانه

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: اكرام ابراهيم عبده محمد
National ID: 26501260100308

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Zoledronic acid for bone protection during hormonal therapy**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين


٢٩٥٠١٢٦٠١٠٠٣٠٨
٢٠٢٤/٠٢
حاصلة على ثانوية عامة
أنثى
مسلمة
البطاقة بمأمورية حتى ٢٠٣١/٠٢/١٠
١


جمهورية مصر العربية
بطاقة تحقيق الشخصية
اكرام
ابراهيم عبده محمد
٢ ش الوزان-درب الجماميز
عابدين - القاهره
١٩٦٥/٠٧/٢٦
٢٩٥٠١٢٦٠١٠٠٣٠٨
KK7148091

XN-L xn

Sample No.: ekram ebrahim
Patient ID:
Name:
Sample Comment:

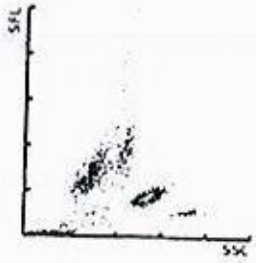
Ward: Adapter: Pos.: 2025/10/02 13:56:34 WB

Doctor:
Birth: Sex:
Nickname: XN-L

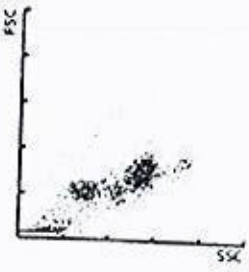
Negative

WBC	3.45	[10^3/uL]		
RBC	4.18	[10^6/uL]		
HGB	12.1	[g/dL]		
HCT	34.8	[%]		
MCV	83.3	[fL]		
MCH	28.9	[pg]		
MCHC	34.8	[g/dL]		
PLT	229	[10^3/uL]		
RDW-SD	41.2	[fL]		
RDW-CV	13.5	[%]		
PDW	9.7	[fL]		
MPV	9.3	[fL]		
P-LCR	18.7	[%]		
PCT	0.21	[%]		
NEUT	1.86	[10^3/uL]	53.9	[%]
LYMPH	1.10	[10^3/uL]	31.9	[%]
MONO	0.38	[10^3/uL]	11.0	[%]
EO	0.09	[10^3/uL]	2.6	[%]
BASO	0.02	[10^3/uL]	0.6	[%]
IG	0.00	[10^3/uL]	0.0	[%]

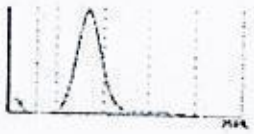
WDF



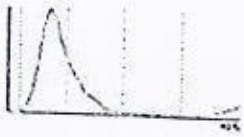
WDF-CBC



RBC



PLT



WBC IP Message

RBC IP Message

PLT IP Message

Name: اكرام ابراهيم عبده محمد **MR. :** 3367	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presented for administration of zoledronic acid (Zometa) and letrozole, with laboratory results reviewed and a request to add paracetamol as needed.

The patient is a 61-year-old receiving luminal-type therapy for estrogen receptor-positive breast cancer with metastatic disease to the lung (pulmonary nodules). Current regimen includes denosumab (Denovo) and zoledronic acid administered every six months, as well as letrozole. A repeat computed tomography of the chest and abdomen/pelvis (CT CAP) is scheduled in three months, with the next visit to occur after the CT CAP report is available.

A CT CAP performed on 14 June 2026 demonstrated no pulmonary, hepatic, or bony metastases. The two previously identified pulmonary nodules remain relatively unchanged, indicating a stationary course. After counseling, the plan is to continue endocrine therapy for an additional three months, followed by repeat CT CAP and bone scan; if the nodules remain stable, the multidisciplinary team will consider surgical intervention.

تحريرا في ,

التاريخ :	20-09-2026	**الوقت :**	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

وظائف كبد وكلى

Tel : Fax :

Name: : اكرم ابراهيم عبده
Patient ID No : 2275-2025
Date Of Birth : Age :
Sex :
DrawDate :

Sample ID :
Hemolyse : Lipemia : Fasting :
Comment 2 :
Doctor :
Run Date : 02/10/2025

Test Name	Result	Units	Flag	Reference Range
ALT S	10.1	U/l		0.0 - 41.0
AST S	23.2	U/l		3.0 - 37.0
BILI.T S	0.48	mg/dl		0.30 - 1.20
BIL D S	0.16	mg/dl		0.00 - 0.40
UREA R	31.2	mg/dl		0.0 - 50.0
CREAT. S	0.93	mg/dl		0.70 - 1.30
U A S	4.72	mg/dl		3.60 - 7.70

--- End Of Report ---

Reviewed by

Date:

Patient Name	: الأستاذه / اكرام ابراهيم عبده محمد	Request No.	: 13003686
Patient No.	: 26501260100308	Request Date	: 21/08/2025
Path No.	: SR-1152/25	Verification Date	: 21/08/2025
Sex	: Female	Financial Category	: حوستف روسي - نفقة الدولة
Age	: 60 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

CamScanner

MRN.	1012526798[illegible]
Patient Name:	IKRAM IBRAHIM ABDO MOHAMED
D.O.B	26/01/1965
IC No.:	26501260100303
Sex	Female
Location.	External Clinic

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id	2508060312	**Collection Date**	06/08/2025 01:35 PM
Accession No	IMM25-3988	**Received Date**	06/08/2025 01:35 PM
Sample Type	Paraffin Block	**Doctor**	Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

4522/25

Clinical Data

Rt. breast cancer

Gross Picture :

Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.
- PR positive in 1-20% of tumor cells is considered as low positive.

(Laterality) breast Cancer:

Right

Diagnosis :

ER: Positive, strong nuclear reaction in 95% of tumor cells (8/8).
PR: Positive, strong nuclear reaction in 35% of tumor cells (7/8).

Signature:

MONA MOHAMED MAMDOUH

Reported and electronically endorsed by
Mona Mohamed Mamdouh Abdellatah

MRN:	10125267986
Patient Name:	IKRAM IBRAHIM ABDO MOHAMMED
D.O.B:	26/01/1965
IC No.:	26501260100306
Sex:	Female
Location:	External Clinic

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2507280035	**Collection Date**	28/07/2025 08:38 AM
Accession No	SUR25-4522	**Received Date**	28/07/2025 08:38 AM
Sample Type	Tissue	**Doctor**	Abd ALLAH Mohamed Ganil

Clinical Information
Right breast 12 O'clock mass, BIRADS 4.
Gross Picture
Multiple cores 0.8-1.2 cm.
Microscopic Picture
A case of invasive duct carcinoma, grade 3. No DCIS component nor lymphovascular emboli were detected. Stroma is desmoplastic with mild inflammatory cell reaction.
Diagnostic Summary
Tumour type:
Invasive duct carcinoma, grade 3.
Laterality

Right
Specimen type:
Tru-cut biopsy.
Location
Lesion Size
Invasive tumor size/s (mm):
Total extent of lesion (mm):
Multiple Lesions
Histological grade
Mitosis score (1-3):

2
Tubular formation score (1-3):

3
Nuclear grade (1-3):

3
Histological grade (1-3):

3
Diagnosis
Right breast/ Tru-cut biopsy:
Invasive duct carcinoma, grade 3.
Comments
ER, PR & HER-2 are available (upon request).
Biopsy reporting category

مركز بهية الهرم

MRN:	1012526/986
Patient Name:	IKRAM IBRAHIM ABDO MOHAMED
D.O.B	26/01/1965
IC No.:	26501260100308
Sex:	Female
Location:	External Clinic

IMMUNOPATHOLOGY

Hormono receptors

Specimen Id	2508060312	**Collection Date**	06/08/2025 01:35 PM
Accession No	IMM25-3988	**Received Date**	06/08/2025 01:35 PM
Sample Type	Paraffin Block	**Doctor**	Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

4522/25

Clinical Data

Rt. breast cancer

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Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

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Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

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- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

Right

Diagnosis :

ER: Positive, strong nuclear reaction in 95% of tumor cells (8/8).

PR: Positive, strong nuclear reaction in 35% of tumor cells (7/8).

Signature

MONA MOHAMED MAMDOUH

Reported and electronically endorsed by
Mona Mohamed Mamdouh Abdelfattah

مركز بهية للورم

Pat. Sex F	Pat. English Name: Ekraam Ibrahim Abdou
Study Date 17/07/2025	Pat. Age 62 Year(s)
Pat. ID: 2017509	Ref. Phy. Dear Prof

Bilateral Mammography High resolution breast US:

Revealed:-

- Glandular Breasts (ACR B).
- The right 12 oclock shows an ill -defined irregular- shaped hypoechoic lesion measures abou 2x1.5 cm.
- Otherwise, both breast parenchyma appear homogenously hyperechoic , no detected other focal lesions .
- No micro-calcification could be detected.
- Normal skin thickness and contour .
- Normal appearance of the areola and nipple.
- Normal duct caliber .
- Bilateral small non specific axillary lymph nodes with preserved fatty hilum .

OPINION:-

- **Right breast lesion with suspicious features as described (BIRADS 4) for pathological correlation.**

Best regards.

DR. Nevien Rashad.

Thanks for your reference.

NB BIRADS (Breast Imaging Reporting And Data System) : 1 Normal , 2 Benign , 3 Probably benign , 4 Probably malignant , 5 Malignant

GUSTAVE ROUSSY INTERNATIONAL EGYPT

MDT FORM

Patient Name	اكرام ابراهيم عبده	Age	60
Sex	F	Special habits	
Family History	-ve	Comorbidity	Hypothyroidism on levothyroxine 50. DM on oral therapy
Surgical History		Telephone	
Menopausal Status	postmenopausal	Marital Status	
ID	2650126010100308	Medical ID	2275/25
Date	28/09/2025	Reason of MDT	

Medical Report

M.D.T

Pathology: IDC GIII
ER 8/8, PR 7/8, Her2: 0

S.M: Rt breast 2'oclock mass 22x11mm
Metsworkup: CT CAP -> few pulmonary nodules largest 11 x 8 mm

Recommendations : MDT requested biopsy for pulmonary nodules -> was not technically feasible.
For ReMDT regarding upfront surgery (give the patient the benefit of doubt)

MDT DECISION	For: after review of radiology → patient is metastatic for FEMARA, ZOMETA/ 6 months

Name	د/ خالد عبدالعزيز	د/ احمد نجيب	د/ رانيا شفيق
Signature			

Name : Ikram Ibrahim Abdu Mohammed
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 03-10-2026 06:54
Medical No. : 475046

NILE DIAGNOSTIC CENTER

Tc-99m MDP SKELETAL SCINTIGRAPHY

History

61 year-old female patient, with history of right breast cancer received hormonal treatment and referred for skeletal survey.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Uniform radiotracer distribution is noticed allover the surveyed skeleton with no evidence of focal active or cold osseous lesion that would account for bone secondaries.
- Degenerative changes are noticed at different levels of the spine and around big joints.

IMPRESSION:-

- No definite osseous metastasis could be depicted.

Much obliged
Dr. Alaa Nagy, MD,
Nuclear medicine